Fortnightly cetuximab + modified FOLFOX-6

NCCP 00733 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Visits, access and care between treatments

Cetuximab and modified FOLFOX6 start on Day 1, followed by one 46-hour fluorouracil infusion with planned disconnection around Day 3. Repeat every 14 days while beneficial and tolerable. No weekly Day 8 cetuximab is scheduled.

STEP 1

Before the first visit

Confirm the exact regimen, tests, medicines, emergency number and treatment goal. Discuss which central line is suitable and arrange pump training.

STEP 2

At the treatment unit

Fortnightly cetuximab infusion time is set from the dose and safe infusion rate; the first dose may take longer. Oxaliplatin and folinic acid are usually infused over about two hours, before fluorouracil. Blood review, preparation, premedicines and observation add time; infusion time is not the total appointment length.

STEP 3

After the infusion

The fluorouracil pump runs for about 46 hours. Confirm where and when it will be disconnected, usually around Day 3; do not disconnect it yourself unless specifically trained.

STEP 4

Between doses

Use the written symptom and support plan; do not wait for the next appointment if urgent symptoms appear.

STEP 5

The next decision

Confirm the next visit after any delay or dose change. Treatment continues only while the oncology team judges benefit and tolerability acceptable. Follow-up mineral checks continue after the final EGFR-antibody dose where applicable.

Transport and daily life

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

If treatment changes

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

Sources: NCCP 00733 v2a · current official regimen checked 5 November 2026. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.